



Policy

Mental Health Missing Persons/Absent Without Leave

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Summary	This policy articulates the processes to be followed when a person accessing mental health services in East Metropolitan Health Service (EMHS) is missing or is Absent Without Leave from an inpatient unit, a community mental health service, general hospital, or Emergency Department.
Policy owner	Executive Director of Mental Health
Contact	Executive Director of Mental Health
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Impact Statement Declaration (ISD)	The Aboriginal consultation is incorporated in this policy for cultural appropriateness and positive health impacts on the Aboriginal Community
Status	Active
Information Classification	OFFICAL



1. Background

1.1 About this document

This policy outlines the processes to be followed when a person accessing mental health services in East Metropolitan Health Service (EMHS) is missing or Absent Without Leave (AWOL), as defined in section 1.2 of this document.

This policy must be read in conjunction with the [Guideline Mental Health Missing Persons/Absent Without Leave](#) on implementing this policy, including risk assessment and risk mitigation.

Note: It is important to note that the definition for a missing person according to the Chief Psychiatrist differs from the definition used by the WA Police Force (WAPOL).

For the purposes of this policy, any person receiving mental health care whose whereabouts are unknown and who is at risk of harm must be reported to WAPOL.

See section 9 for further information about involving WAPOL.

1.2 Key definitions

Term	Definition
Absent Without Leave (AWOL)	Under section 97 of the Mental Health Act 2014 (WA) (MHA), a person is considered Absent Without Leave (AWOL) from an authorised hospital or other place (such as a clinic) if they leave without permission, when they are: 1) detained under the MHA: a. to be taken (transported) to an authorised hospital or other place (ie. the person is under Form 4A); or b. for assessment or examination (ie. once the person is received in the authorised hospital or other place and are under Form 1A or 3A); or 2) under an inpatient treatment order (ie. the person becomes an involuntary inpatient and under Form 6A) and: a. leave has not been granted; or b. they fail to return to the hospital after their leave has been cancelled or expired; or 3) an involuntary community patient who is ordered to attend a place as a result of breaching their community treatment order (CTO) (ie. person is under Form 5F) and is required to be detained at that place (see s 130(2)(b) MHA).

Missing person	The Office of the Chief Psychiatrist Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist – Policy for Public Mental Health Services defines a ‘missing person’ as “any voluntary psychiatric patient with risk concerns for harm, who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.”
Harm	As per the Department of Health’s Mental Health Consumers Who Are Missing or Absent Without Leave Policy and the OCP Mandatory Reporting of Notifiable Incidents Policy , harm is defined as: harm to self, harm to others, harm from others, and harm to property, such as arson.

Refer to: [Chief Psychiatrist Clinicians’ Practice Guide to the Mental Health Act 2014](#) (section 4.10)

2. Risk assessment and risk mitigation

Risk assessment and risk mitigation must be completed in alignment with the Best Practice Guideline: [Mental Health Consumers Who Are Missing or Absent Without Leave](#)

3. Determining the person’s risk of harm when missing or AWOL

As soon as it becomes apparent that a person receiving care is missing or AWOL, staff members should first identify any concerns regarding risk of harm to that person and/or others, in order to determine the appropriate course of action.

Considerations must include (but are not limited to):

- The person’s status under the MHA (voluntary or involuntary)
- Consideration of the person’s clinical history, including the existing care plan and risk management plan
- Their mental state and risk concerns at the last point of contact, and
- Collateral information from a support person.

4. Inpatient mental health services

Inpatient ward staff must take all reasonable steps to know the whereabouts of the people in their care at all times.

Note: EMHS Services/Hospitals with the use of Closed-Circuit Television (CCTV) within their premises, which are monitored by Security Services, should consider

information from this media as a means of locating the person on site. This information should be considered by the Manager.

Refer to: EMHS [Use of Video Monitoring in Mental Health Inpatient Units](#) Policy

4.1 Immediate actions

4.1.1 Suspected Missing or AWOL

Regardless of legal status (voluntary or involuntary), the following immediate steps should be taken if a person's whereabouts are unknown and it is reasonably suspected the person is missing or AWOL:

- The staff member (regardless of discipline) who suspects that the person is missing or AWOL is to inform the Shift Coordinator immediately.
- The Shift Coordinator is to inform the Nurse Unit Manager or relevant senior nurse, the treating doctor or duty doctor, and Security and immediately organise for a thorough search of the inpatient unit and surrounds.

4.1.2 Once Confirmed the Person is Missing or AWOL, with Identified Risk Concerns

- Attempt to make contact with the person and document these attempts in their medical record.
- Inform the person's carers and support people about the unauthorised absence and ask for information on their possible whereabouts, and keep them updated at regular intervals as the situation changes (i.e., if the person is located).
- Notify WAPOL, including a description of:
 - The person
 - The clothing being worn when they were last seen
 - Any distinctive or distinguishing features
 - The mental health staff's concerns about level and nature of risk of harm.

Note: When a person receiving voluntary care goes missing, WAPOL should only be notified if they have assessed risk concerns regarding harm to themselves, harm to or from others, and harm to property (e.g., arson).

- Notification details for WAPOL are to be documented on the appropriate site-based form (e.g. Missing Patient, Absent Without Leave) which is to be signed by the relevant doctor or the Nurse Unit Manager and faxed to the police.
- Contact the Community Treatment Team (CTT) or Assessment and Treatment Team (ATT) (as appropriate) with a request to accompany police (if

required) to assess the person and change their status (under the MHA) if applicable.

- Inform the person's Care Coordinator (where allocated).
- Add an alert to PSOLIS and WebPAS about the incident.
- Notify the Chief Psychiatrist, by entering the incident into Datix Clinical Incident Management System (CIMS) as per the [OCP Mandatory Reporting of Notifiable Incidents Policy](#).

4.1.3 Additional Requirements if the Person is under the MHA and is AWOL

- The Medical Co Director/Head of Department (or delegate) is to be notified by the treating psychiatrist of any person receiving care who is AWOL and is to be informed when the person is located and/or returned.
- The Medical Co-Director/Head of Department is responsible for informing the Executive Director Mental Health
- **If after-hours:** the site Executive on call is responsible for informing the Executive Director Mental Health

Refer to: MP 0099/18 [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#)

4.1.4 Immediate Actions for Person with no Identified Risk Concerns whose whereabouts are unknown

- Attempt to contact them to try to determine their whereabouts, and document these attempts in their medical record.
- Contact carers and support people and inform them of the situation at the soonest possible time.
- Inform the person's Care Coordinator (where allocated).
- If they cannot be located after reasonable attempts to locate them, a decision may be made to discharge the person receiving voluntary care if they are not considered to have risk concerns for themselves or others.
- Carers and support people are to be informed of the decision to discharge the person.
- Add an alert to PSOLIS and WebPAS about the incident.

Refer to: Department of Health [Best Practice Guideline: Mental Health Consumers Who Are Missing or Absent Without Leave](#)

4.2 Ongoing Legal Status

- If the person receiving care under the MHA remains AWOL, clinical review of the case is required by the consultant psychiatrist.
- If the person is returned to the inpatient unit, they will be reassessed.
- During the period whilst the person is AWOL, they will retain their involuntary status under the MHA unless they are discharged.
- This AWOL status will remain until they are located and reviewed by the consultant psychiatrist, or until the MHA Form authorising their detention expires.

4.3 After Hours/Out of Hours

If a person receiving inpatient care is noted as missing or AWOL out of business hours, a notification is to be made to MHERL and ATT with a request to WAPOL (if required) to locate and assess the person.

5. Community mental health services

When a staff member becomes aware that a person accessing community mental health services is suspected to be missing, or has missed an appointment and it is determined by the treating team that this is of concern, the staff member is to take appropriate steps based on an assessment of the person's risk profile and obtain information on their whereabouts and wellbeing. For information about actions to take when someone who is on a Community Treatment Order (CTO) goes missing, refer to section 7.

5.1 Immediate Actions for Missing Person with Identified Risk Concerns

- Follow local site procedures for non-attendance (see below).
- Following discussion with the multidisciplinary team including senior staff/the consultant psychiatrist, notify WAPOL where it is deemed appropriate.
- Add an alert to PSOLIS and WebPAS about the incident.

5.2 Immediate Actions for Person with no Identified Risk Concerns (neither AWOL or missing)

- Follow local site procedures for non-attendance (see below).
- WAPOL is not to be contacted.

Refer to:

- [AKG Scheduling, Rescheduling and Non-Attendance \(DNA\) of Appointments in Community Mental Health Clinics Procedure](#)
- [RPBG Community Mental Health Services Triage to Discharge SOP](#)

6. Community Treatment Order

A person on a Community Treatment Order (CTO; on a Form 5A under the MHA) is considered AWOL if:

- they are non-compliant with the CTO conditions, resulting in an 'Order to Attend' a particular place to receive treatment (under Form 5F),
and
- the person fails to attend or leaves the place identified in the Order to Attend before receiving treatment.

Where a person is not AWOL but is suspected missing, and is not in breach of their CTO, refer to Section 4 of this document.

6.1 Immediate Actions for Person on a CTO Confirmed as AWOL

- Follow local site procedures for non-attendance and breach of a Community Treatment Order.
- Following discussion with the multidisciplinary team including senior staff/the consultant psychiatrist, notify WAPOL and/or make a Form 4A Transport Order as appropriate (see [Clinician's Practice Guide to the Mental Health Act 2014](#) section 5.10.8)
- Notify the Chief Psychiatrist by entering the incident into Datix Clinical Incident Management System (CIMS).
- Add an alert to PSOLIS and WebPAS about the incident.

6.1.2 Immediate Actions for Missing Person in Breach of CTO but not AWOL

Follow local site procedures for non-attendance and breach of a Community Treatment Order.

Refer to: [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014: Community Treatment Orders \(Chapter 5\)](#)

7. Emergency Departments/General Health Wards

For a person accessing services in an EMHS Emergency Department or General Health ward who is missing or AWOL, staff should follow local site procedures.

Refer to:

- [EMHS Discharge Against Medical Advice Policy](#)

- [Management and Review of 'Did Not Wait' Patients that Present to Emergency Departments](#)

8. Involving the WA Police Force (WAPOL)

In the context of this policy, WAPOL may be engaged to assist in the following scenarios:

- Where a person receiving care is AWOL (in emergency situations and/or where an Apprehension and Return Order is made); and
- Where a person receiving care is a missing person.

Where appropriate, community mental health service clinicians may be asked to visit the residence of the person who is missing or AWOL. If there are risk concerns identified (imminent or actual harm to person/others), WAPOL is to be asked to conduct a police welfare check and accompany the community mental health clinicians. The referral to WAPOL for a welfare check does not replace a mental health risk assessment and mental state examination by a mental health clinician.

In Emergency situations (regardless of legal status):

Notify WAPOL by dialling 000

In non-emergency situations (including an Apprehension and Return Order):

Notify WAPOL by dialling 131 444

When Engaging with WAPOL, staff must:

- Assess and clearly communicate the risk of harm to the person receiving care and/or others
- Share any risk factors for consideration
- Recognise the terminology used by WAPOL (as below) and ensure mutual understanding of the situation, and
- Keep police aware of any changes in risk.

8.1 WAPOL Terminology

It is important to note that the definition for a missing person according to the Chief Psychiatrist differs from the definition used by the WA Police Force (WAPOL). Refer to section 1.2 for OCP definitions.

Absent (WAPOL Definition)

- A person whose whereabouts are unknown
- there are no genuine and serious concerns for the person's safety and welfare and no suspicion of criminality surrounding their absence; and
- they are simply not at a place where they are expected or required to be.

Missing (WAPOL Definition)

- A person whose whereabouts are unknown
- police and the reporting person have formed a reasonable belief supported by the available evidence that there is a genuine and serious concern for the person's welfare and safety, and/or
- there is suspicion of criminality surrounding their disappearance (i.e. abduction/homicide).

Refer to: MP 0012/16 [Procedures for Involving Police When Mental Health Consumers Are Missing or Absent Without Leave](#)

9. Location, Transport and Safe Return of People Receiving Care

There are provisions under Section 98 of the MHA to apprehend and return a person who is AWOL.

Staff are to refer to the following policies:

- MP0063/17 [Road-Based Transportation for Mental Health Consumers Policy](#) and
- [Mental Health Patient Transport Risk Rating Form](#)
- EMHS [Transportation of Mental Health Consumers](#) Policy
- [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014: Transport Orders \(Part 10\)](#)

If a person receiving care who is missing is located and returned by mental health service staff, or returns to the inpatient unit themselves, the Shift Coordinator/Team Leader is to notify:

- The person's carers and support people
- The person's treating doctor
- Security services
- WAPOL (if notified of their absence)

9.1 Identification and Assessment

Immediately on return to the ward a formal identification and assessment of the person's physical and mental state is to be conducted on return to the service.

The formal identification is to occur using photographic identification or by two clinicians who know the person. The identification and assessment outcome is to be clearly documented in the person's healthcare record.

The senior ward nurse is to ensure a face-to-face assessment is completed by a senior mental health clinician. The assessment is to include all of the following:

- A mental state examination and risk assessment
- Physical appearance
- Observations/investigations as necessary e.g. Blood Alcohol Level/Urine Drug Screen
- Identification of any specific reason/s for going missing from the ward

The senior mental health clinician is to discuss with the responsible medical officer the outcomes of the assessment and the resulting care plan. All assessments, plans and decisions are to be clearly documented in the person's healthcare record.

Refer to: EMHS [Patient Identification and Procedure Matching](#) Policy.

10. Reporting and investigation obligations

All incidents (or near misses) where people accessing mental health services are AWOL or missing are to be entered as an incident into the Datix Clinical Incident Management System (CIMS). This requirement meets the obligation to report these events as notifiable incidents to the Chief Psychiatrist as per [EMHS notifications requirements under the WA Mental Health Act 2014 Policy](#), [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#), and [MP 0122/19 Clinical Incident Management Policy](#).

See [Mental Health Missing Persons Guideline](#) for further information.

11. Death of a person receiving care

Where staff become aware of the death of a missing person, staff are to follow existing policies and procedures.

Refer to:

- EMHS [Reporting and Care of the Deceased](#) Policy
- [Coroners Act 1996: Reporting of Deaths](#)

- Department of Health [Death in Hospital Form](#)
- [MP 0122/19 Clinical Incident Management Policy](#)

12. Training and Education

EMHS Sites must provide appropriate training and professional development for all health professionals, including security guards, in managing AWOL and missing persons. Training should include:

- Reducing the risk of AWOL and missing person incidents
- Definition of AWOL and missing person
- Appropriate diligent efforts to locate people who are AWOL or a missing person
- Responding effectively to AWOL and missing person incidents
- Appropriate involvement of people receiving care and their carers and support people
- Communication, collaboration and information sharing with the WA Police Force and other relevant agencies
- Notifications
- Reporting and record-keeping
- Culturally appropriate care and trauma-informed approaches.

13. Compliance monitoring and legislative penalties

13.1 Compliance monitoring

The EMHS Executive Director Mental Health is responsible for ensuring EMHS mental health staff are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

Compliance monitoring methods include the following:

- Number of reported incidents of missing people from EMHS mental health services.
- Number of incidents reported to the Chief Psychiatrist.
- Missing person documentation audit.

- Annual thematic reviews of all reported AWOL and missing person incidents to identify common contributory factors or trends and determine whether any systemic factors could be addressed to mitigate the risk of future incidents.

13.2 Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

14. Legislation

- Mental Health Act 2014 (WA)
- Mental Health Regulations 2015 (WA)

15. Additional references

15.1 WA Health policies

- [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#)
- [MP 0012/16 Procedures for Involving Police When Mental Health Consumers Are Missing or Absent Without Leave](#)
- [MP0122/19 Clinical Incident Management Policy](#)
- [Clinical Incident Management Guideline](#)
- [MP0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#)
- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [MP0063/17 Road-Based Transportation for Mental Health Consumers Policy](#)
- MP 0099/18 [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#)

15.2 EMHS policies

- [Care Coordination in Mental Health Services](#)
- [Patient Identification and Procedure Matching](#)
- [Reporting and Care of the Deceased](#)
- [Transportation of Mental Health Consumers](#)
- [Use of Video Monitoring in Mental Health Inpatient Units](#)
- [Notifications requirements under the WA Mental Health Act 2014](#)
- [Mental Health Missing Persons without leave Guideline](#)

15.3 Other

- [Death in Hospital Form](#)
- [Stokes, B \(2012\). Review of the admission or referral to and the discharge and transfer practices of public mental health facilities/services in Western Australia. Perth, Government of Western Australia, Department of Health and Western Australian Mental Health Commission](#)
- [Best Practice Guideline: Mental Health Consumers Who Are Missing or Absent Without Leave](#)
- [Triage to Discharge - Mental Health Framework for State-wide Standardised Clinical Documentation](#)
- [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- [Mosel, K. A., Gerace, A., & Muir-Cochrane, E. \(2010\). Retrospective analysis of absconding behaviour by acute care consumers in one psychiatric hospital campus in Australia. International journal of mental health nursing, 19\(3\), 177–185.](#)